Challenge Stigma / Find Peace

*FrontlineFolks | Guide #35*

# Sex After Injury or Major Health Change

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*A Field Guide for Frontline Folks and Their Families*

Field notes for physical limitations, pain, adaptation, and partnered problem-solving.

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## Disclaimer

This guide is for general education only.

It is not:

- Therapy
- Medical care
- A crisis service
- Individualized advice

Reading this guide does not create a therapist–client relationship with the author or with Never Forgotten Services, PLLC.

If you are in crisis or thinking about harming yourself or someone else, call 988 in the United States, go to your nearest emergency room, or contact your local emergency services.

If you want mental health treatment, please contact a licensed provider in your state or call the number on the back of your insurance card.

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## 1. What Is Happening

Your body changed. Maybe you had an injury, surgery, or a diagnosis that limits what you can do physically. Sex was a certain way before, and now positions hurt, pain is unpredictable, or you simply can't do what you used to. You're grieving the way sex used to work. You might be avoiding it entirely because you're afraid of pain or failure. Your partner doesn't know how to help—they don't want to hurt you, but they also miss sexual connection. This isn't about willpower or desire. It's about your body having new constraints and needing a new approach.

***Common signs***

- Pain during or after sex: Certain positions, depths, or duration causes pain—during or hours later.
- Difficulty with arousal or orgasm: Pain, medication, or changed nerve sensation makes arousal difficult.
- Fear of pain limiting exploration: You avoid trying new things because you're afraid it will hurt.
- Asymmetrical desire: One partner wants sex more, the other is cautious about pain or limitation.
- Difficulty with penetration: Specific positions or types of penetration are no longer possible.
- Fatigue making sex impossible: Energy limits mean sex has to happen at specific times or not at all.
- Loss of spontaneity: Sex requires planning, positioning, or medical considerations (pain medication timing, etc).
- Feeling like a burden: You believe your injury/illness is making sex worse for your partner.

> **This is manageable, not permanent:**
> Physical limitations don't mean the end of sexual intimacy. They mean it has to be reimagined, not abandoned.

## 2. Why It Escalates

Injury or health change creates physical constraints (pain, limited range, reduced sensation) AND psychological ones (fear, grief, shame). Sex requires vulnerability and risk-taking. If your body hurt you during sex last time, your nervous system gets cautious. Add partner confusion (they don't want to hurt you, but they miss sex), and you get avoidance. The longer you avoid, the more that becomes the new normal.

***Common amplifiers***

- Fear of pain limiting exploration: One painful experience creates catastrophic thinking: 'It will always hurt.'
- Partner confusion becoming resentment: They don't know what's safe to try, so they stop trying, you feel rejected.
- Grief compounding: Loss of the body you had + loss of the sex you had = doubles the grief.
- Avoidance escalating: You haven't had sex in months. Partner feels rejected. Talking about it feels harder.
- Medical and sexual disconnect: You talk to your doctor about pain, your partner about missing connection, but no one puts them together.
- Medication side effects: Some pain meds reduce libido or sexual response, which gets blamed on injury when it's the medication.
- Disability grief: Sex was something your pre-injury body could do. This new body requires adaptation, and that feels unfair.

> *Why this works: reducing total load and increasing predictability lowers baseline arousal and improves regulation.*

## 3. What To Do This Week

You can adapt sex to your new physical reality and rebuild intimacy through collaboration.

1. Get specific medical guidance: Talk to your doctor about safe sexual activity post-injury. Ask: 'What positions are safe? What pain medication timing matters? What should I avoid?'
2. Identify pain-free zones and positions: Experiment slowly to find what doesn't hurt. Start there and build from what works.
3. Separate pain from failure: Pain during sex isn't failure. It's information. 'This position causes pain' doesn't mean you can't have sex, it means try a different position.
4. Rebuild communication with partner: 'I want sex, but I need us to figure out what works now. This is a problem we solve together, not a failure.'

5. Try non-traditional sexual expression: If penetration is painful, manual stimulation, oral sex, or other forms become primary. Pleasure doesn't have one shape.
6. Plan for energy/pain management: Schedule sex at times when you're rested and pain medication is working. Predictability reduces anxiety.
7. Address medication side effects: If libido dropped with a new medication, talk to your doctor about alternatives or timing.
8. Practice partnered problem-solving: 'What works for you? What works for me? How do we both feel good?' is a different conversation than 'Can you still want me?'
9. Grieve the loss: Your old sex was real and good. The new version will be different, not lesser. Both can be true.

> Field rule: set the bar low enough to do it even on your worst day.

## 4. For Partners and Team

Stress communication should be clear, brief, and repeatable. Predictable routines protect relationships.

- Use concrete language: 'This position causes pain. Let's try this one instead.'
- Make it collaborative: 'I want sex with you. We need to figure out what works for both of us now.'
- Name the grief: 'I miss how sex used to be. I'm also grieving my body. And I still want intimacy with you.'
- Offer specific information: Tell them what hurts, what feels good, what you need. Don't make them guess.

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## 5. When To Get More Help

Self-guided tools are a start. Escalate care when symptoms persist, spread, or begin harming safety and function.

- Pain during sex is ongoing and limiting your sexual frequency significantly.
- You've stopped having sex entirely due to fear of pain.
- Partner is becoming resentful or considering ending the relationship.
- Medication side effects are impacting arousal/pleasure and it's not improving.
- Relationship conflict around sex is affecting your overall connection.

- Sex therapy (especially with a therapist educated in pain management) can help couples adapt sex to new physical realities.

## 6. 7-Day Field Drill

Use the worksheet every day for one week. Track trigger, body signal, and reset action. Keep what works and remove what does not.

> **Daily drill:**
> 1) Name the trigger.
> 2) Name the first body signal.
> 3) Run one reset action.
> 4) Log response quality.

# Worksheet 1: Worksheet: Sex After Injury or Major Health Change

Use this to track the pattern in this chapter and build a practical response you can repeat under load.

### Where did this pattern show up this week?

*List specific moments at work, at home, and in transition.*

### What did it cost you when it took over?

*Name impact on body, mood, decisions, and relationships.*

### What two actions will you run for the next 7 days?

*Choose actions small enough to use on hard days.*

**1. Trigger:**

*What activated the pattern?*

**2. Early body signal:**

*What did your body do first?*

**3. Reset action used:**

*What fast action lowered intensity?*

**4. Better next response:**

*What exact behavior will you use next time?*

> Track this daily for one week, then tighten the plan around what actually worked.

# Worksheet 2: Worksheet: Sex After Injury or Major Health Change - Partner/Team Check-In

Use this page for one short practical check-in each week.

### What do others notice first when this pattern is active?

*Capture observable behavior, not labels.*

### What support helps most and least?

*List what helps and what escalates the issue.*

### What agreement will you both practice this week?

*One clear action from each side.*

### 1. Signal phrase:

*What phrase means pause and reset?*

**2. Time-to-return commitment:**

*When will re-engagement happen?*

**3. Repair step:**

*What specific repair happens after a rupture?*

> Keep check-ins short, specific, and repeatable.

## Closing

You've seen and carried more than most people will ever know.

Blow ups and shutdowns are not proof that you're broken or that your relationship is doomed.

They are signs that your nervous system has been running in survival mode for a long time.

The fact that you picked up this guide and read it matters.

You are choosing to be intentional about how you show up for the people who matter to you.

That choice is a lot more powerful than having flawless days.

## About the Author

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Michael is a U.S. Army combat veteran and licensed clinical social worker who specializes in working with military, veterans, and first responders—our frontline folks. After leaving the military, he spent years fighting his own demons and trying to figure out why he could hold it together on the job but feel completely wrecked at home. The turning point was realizing he wasn't the only one living that split life—and that nobody should have to feel that alone, confused, or broken by what the job asks of them.

Since then, his work has focused on trauma, the impact of high-stress jobs on home life, and helping families make sense of what the job brings home. He uses plain language, zero shame, and a "you're not crazy, this actually makes sense" approach to help frontline folks and their families understand what's happening in their nervous systems and learn how to move through the chaos together.